ISSUED, VALID FROM 30 SEPTEMBER 2026

Urinary Tract Infection in Women aged 16 to 64

Patient Group Direction, version 010, issued 30 September 2026. Two medicines: nitrofurantoin first line, trimethoprim only where nitrofurantoin is unsuitable.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Scope and the order of choice

This PGD authorises the supply of an antibiotic for the treatment of uncomplicated lower urinary tract infection in non-pregnant women aged 16 to 64 years.

- **NITROFURANTOIN IS FIRST LINE. Supply trimethoprim only where nitrofurantoin is unsuitable for this patient, and record the reason.**
- Trimethoprim resistance in E. coli is common and is not predictable at the counter. Nitrofurantoin retains high urinary activity and is the NICE first choice.
- **ONE COURSE PER EPISODE. If the patient has already had an antibiotic for this same episode, from anyone, do not supply a second course. Refer.**
- **Ask the red flag questions in Appendix 1 before supplying anything. If any is present, refer and do not supply.**
- **Give 48 hour safety netting to every patient and record that you did.**
- **This PGD does not cover men, children, pregnant or breastfeeding women, catheterised patients, recurrent UTI, or any patient with signs of upper urinary tract infection. Refer.**
- This PGD does not cover prophylaxis or the prevention of recurrent infection.

Recurrent UTI: the definition that applies

- **Recurrent UTI means 2 or more episodes in the last 6 months, or 3 or more in the last 12 months. Ask both questions and record both answers.**
- A patient meeting that definition is EXCLUDED from this PGD and referred for investigation, whatever their current symptoms.

Sexually transmitted infection

- Vaginal discharge, pelvic pain, intermenstrual or post-coital bleeding, or a new or recent sexual partner should raise chlamydia, gonorrhoea and trichomonas as alternative diagnoses.
- **Where discharge is present, or the history raises the possibility, do not supply under this PGD. Refer for testing.**

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Nitrofurantoin, first line

Summary of NICE guidance for lower urinary tract infection

Urinary tract infection (lower): antimicrobial prescribing, NICE guideline NG109, published 31 October 2018; minor updates July 2019 (nitrofurantoin formulations), May 2022 (recommendation 1.2.1) and May 2025 (links to MHRA advice on nitrofurantoin). Summarised 9 September 2026.

What the guideline covers and assessment

NG109 sets out an antimicrobial prescribing strategy for lower urinary tract infection (also called cystitis) in children, young people and adults who do not have a catheter.

NICE describes lower UTI as an infection of the bladder usually caused by bacteria from the gastrointestinal tract entering the urethra and travelling up to the bladder.

The guideline does not set out a diagnostic list of lower UTI symptoms; it directs that if there are symptoms of pyelonephritis (such as fever) or a complicated UTI, the NICE guideline on acute pyelonephritis should be followed for antibiotic choices.

When deciding on treatment, take account of the severity of symptoms, the risk of complications (higher in people with known or suspected structural or functional abnormality of the genitourinary tract or immunosuppression), previous urine culture and susceptibility results, previous antibiotic use which may have led to resistant bacteria, and the person's preferences.

Give self-care advice to all people with lower UTI.

Back-up versus immediate antibiotic in non-pregnant women

For women with lower UTI who are not pregnant, consider either a back-up antibiotic prescription (to use if symptoms do not start to improve within 48 hours or worsen at any time) or an immediate antibiotic prescription.

NICE notes that the evidence for back-up prescriptions was only in non-pregnant women with lower UTI where immediate antibiotic treatment was not considered necessary.

When a back-up prescription is given, advise that an antibiotic is not needed immediately, to use the prescription if symptoms do not start to improve within 48 hours or worsen at any time, and about possible adverse effects, particularly diarrhoea and nausea.

When an immediate prescription is given, advise about possible adverse effects (particularly diarrhoea and nausea) and about seeking medical help if symptoms worsen rapidly or significantly, do not start to improve within 48 hours of taking the antibiotic, or the person becomes systemically very unwell.

Antibiotic choice for non-pregnant women aged 16 and over

First choices: nitrofurantoin (if eGFR is 45 ml/minute or more) 100 mg modified-release twice a day (or, if unavailable, 50 mg four times a day) for 3 days, or trimethoprim 200 mg twice a day for 3 days if there is a low risk of resistance.

A lower risk of trimethoprim resistance is more likely if trimethoprim has not been used in the past 3 months, previous urine culture suggests susceptibility, and in younger people in areas where local data suggest resistance is low; a higher risk is more likely with recent use and in older people in residential facilities.

Second choices (if no improvement on the first choice taken for at least 48 hours, or when the first choice is not suitable): nitrofurantoin as above for 3 days if not already used, pivmecillinam 400 mg initial dose then 200 mg three times a day for a total of 3 days, or fosfomycin 3 g as a single dose sachet.

Take account of local antimicrobial resistance data, check any previous urine culture and susceptibility results and antibiotic prescribing, and see the BNF for use in hepatic or renal impairment and breastfeeding.

Nitrofurantoin may be used with caution if eGFR is 30 to 44 ml/minute only for uncomplicated lower UTI caused by suspected or proven multidrug-resistant bacteria and only if potential benefit outweighs risk; NICE also links to the MHRA advice on monitoring for pulmonary and hepatic adverse reactions to nitrofurantoin.

If a urine sample has been sent for culture, review the antibiotic choice when results are available and change it if the bacteria are resistant and symptoms are not already improving, using a narrow-spectrum antibiotic wherever possible.

Pregnant women, men and children are managed differently

Offer an immediate antibiotic prescription to pregnant women and to men with lower UTI, and obtain a midstream urine sample before antibiotics are taken for culture and susceptibility testing.

For pregnant women aged 12 years and over, the first choice is nitrofurantoin (if eGFR is 45 ml/minute or more) 100 mg modified-release twice a day (or 50 mg four times a day) for 7 days, avoided at term because it may produce neonatal haemolysis; second choices are amoxicillin 500 mg three times a day for 7 days (only if culture results are available and susceptible) or cefalexin 500 mg twice a day for 7 days.

Asymptomatic bacteriuria is treated with antibiotics in pregnant women because it is a risk factor for pyelonephritis and premature delivery, but is not routinely screened for or treated in non-pregnant women, men, young people and children.

For men aged 16 years and over, the first choices are trimethoprim 200 mg twice a day for 7 days or nitrofurantoin (if eGFR is 45 ml/minute or more) 100 mg modified-release twice a day for 7 days; nitrofurantoin is not recommended where prostate involvement is suspected because it is unlikely to reach therapeutic levels in the prostate.

For men with no improvement on first choice, consider alternative diagnoses and follow the NICE guidelines on acute pyelonephritis or acute prostatitis, basing antibiotic choice on recent culture and susceptibility results.

For children and young people under 16, obtain a urine sample before antibiotics, offer an immediate antibiotic prescription, and follow the NICE guideline on urinary tract infection in under 16s; children under 3 months should be referred to a paediatric specialist.

Reassessment and referral

Reassess if symptoms worsen rapidly or significantly at any time, or do not start to improve within 48 hours of taking the antibiotic.
At reassessment take account of other possible diagnoses, any symptoms or signs suggesting a more serious illness or condition such as pyelonephritis, and previous antibiotic use.
Send a urine sample for culture and susceptibility testing at reassessment if this has not already been done, and review treatment when results are available.
Refer people aged 16 years and over with lower UTI to hospital if they have any symptoms or signs suggesting a more serious illness or condition (for example, sepsis).
Refer children or young people to hospital in line with the NICE guideline on urinary tract infection in under 16s, and assess and manage children under 5 who present with fever as outlined in the NICE guideline on fever in under 5s.

Self-care advice

Advise people with lower UTI about using paracetamol for pain, or if preferred and suitable, ibuprofen.
Advise people about drinking enough fluids to avoid dehydration.
NICE states that no evidence was found on cranberry products or urine alkalinising agents to treat lower UTI.

Patient Group Direction for the supply of nitrofurantoin 100mg modified release capsules for the treatment of uncomplicated lower urinary tract infection in non-pregnant women aged 16 to 64 years.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the product supplied, the BNF, and the NICE guideline on lower urinary tract infection in women (NG109) and NICE CKS.
- All users must be able to distinguish a lower urinary tract infection from pyelonephritis, and must know the red flags in Appendix 1 without looking them up.
- All users must be competent to take a sexual history sufficient to consider a sexually transmitted infection as an alternative diagnosis, and to refer for testing without embarrassment on either side.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these medicines.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of uncomplicated lower urinary tract infection in a non-pregnant woman aged 16 to 64 years, presenting with two or more of dysuria, new nocturia, urinary frequency or urgency, in the absence of any red flag in Appendix 1. This is the first line medicine under this PGD.
Inclusion criteria	● Female, aged 16 to 64 years inclusive. ● **Two or more of: dysuria, new nocturia, urinary frequency or urgency. Where only one symptom is present, refer rather than supply.** ● Symptoms present for 7 days or fewer. ● **No red flag in Appendix 1 present.** ● No antibiotic already taken for this episode. ● Renal question answered as set out below; where aged 60 to 64, an eGFR of 45 mL/min or more within the last 12 months seen by the pharmacist and recorded. This eGFR requirement applies to nitrofurantoin only. ● Able to give valid informed consent, and consent given. ● No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following applies.** ● **Male. Any UTI in a man is complicated by definition and needs assessment.** ● Aged under 16 or 65 and over. ● **Pregnant, possibly pregnant, or breastfeeding. Ask; do not assume. Nitrofurantoin is also avoided at term because of neonatal haemolysis.** ● **ANY red flag in Appendix 1: fever, rigors, loin or flank pain, nausea or vomiting, visible haematuria, confusion or new**

	drowsiness, or the patient appearing systemically unwell. ● **Known kidney disease; or, in a patient aged 60 to 64, no eGFR result of 45 mL/min or more dated within the last 12 months seen by the pharmacist and recorded. The eGFR requirement applies to nitrofurantoin only. See the renal row below.** ● **An indwelling urinary catheter, or a catheter removed within the last 7 days. Catheter-associated infection is not uncomplicated UTI and does not respond to a 3 day course.** ● **Recurrent UTI, meaning 2 or more episodes in the last 6 months or 3 or more in the last 12 months.** ● **An antibiotic already taken for this same episode, whether supplied here, by a GP, or obtained elsewhere. One course per episode. Refer.** ● Symptoms present for more than 7 days. ● Known hypersensitivity to nitrofurantoin or to any excipient in the product supplied. ● **Glucose-6-phosphate dehydrogenase (G6PD) deficiency. Nitrofurantoin causes haemolysis. Ask where the patient or their family originates from an area where G6PD deficiency is common.** ● Previous pulmonary reaction, peripheral neuropathy or hepatic reaction attributed to nitrofurantoin. ● Acute porphyria. ● **Vaginal discharge, pelvic pain, intermenstrual or post-coital bleeding, or a history suggesting a sexually transmitted infection. Refer for testing.** ● Known structural or functional abnormality of the urinary tract, renal stones, or immunosuppression. ● Valid informed consent not obtained.
Renal function, how to assess it here	**Ask the patient directly: "Have you ever been told you have kidney disease, or that your kidneys do not work as well as they should?"** ● Answer YES, or the patient is under any renal follow-up: EXCLUDE. Refer. ● Answer NO and aged 16 to 59: proceed. ● **Answer NO and aged 60 to 64: proceed only where an eGFR of 45 mL/min or more, dated within the last 12 months, has been seen by the pharmacist (NHS App, GP summary record or a letter) and the result, its date and where it was seen are recorded. Where no such result can be seen, or the result is below 45 mL/min or more than 12 months old: EXCLUDE. Refer for a renal function check first. This eGFR requirement applies to the nitrofurantoin arm only; the trimethoprim arm states its own renal rule.** **The patient does not know, at any age: EXCLUDE. Refer for a renal function check first.**
Cautions	Diabetes, and any condition causing peripheral neuropathy, increase the risk of nitrofurantoin-associated neuropathy. Counsel on new numbness or tingling and to stop and seek advice. Advise the patient the medicine must be taken with food or milk, both to reduce nausea and because absorption is better. Warn that urine may turn dark yellow or brown. This is harmless and expected. Advise the patient to complete the full 3 day course even if symptoms settle sooner. Encourage fluids and offer paracetamol, or ibuprofen if preferred and suitable, for pain as a separate pharmacy sale, subject to the usual checks.

Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Direct to the GP the same day where a red flag is present, or to NHS 111 or urgent care out of hours. Give the 48 hour and immediate-action advice regardless of whether anything is supplied. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.
Safety netting, to be given to every patient	**48 HOURS. Tell the patient, in these terms: "If you are no better in 48 hours, or you get worse at any point, contact your GP or NHS 111 the same day. Do not wait."** **Tell the patient to seek help IMMEDIATELY, not in 48 hours, if any of the following develops:** ● A temperature, shivering or shaking uncontrollably. ● Pain in the back or side, below the ribs. ● Feeling sick or being sick. ● Blood in the urine that they can see. ● Confusion, drowsiness, or feeling very unwell in themselves. ● Any new pain or bleeding in pregnancy, if pregnancy is a possibility.

The medicine

Name, form and strength	Nitrofurantoin 100mg modified release capsules.
Legal category	POM.
Dose and frequency	100mg twice daily for 3 days.
Quantity supplied	6 capsules. A 3 day course. No repeat supply under this PGD.
Route and method	Oral, with food or milk.
Maximum treatment period	3 days. One course per episode. A patient still symptomatic at 48 hours is referred, not re-supplied.
Storage	Store in the original packaging below 25 degrees Celsius.
Drug interactions	Antacids containing magnesium trisilicate reduce absorption; separate them. Probenecid and sulfinpyrazone reduce urinary excretion and so reduce efficacy while increasing toxicity. Quinolones are antagonised. Check the current SPC and BNF for the patient's full medicine list.
Adverse effects	Common: nausea, headache, flatulence, diarrhoea, dark or brown urine. Uncommon: rash, pruritus. Rare but serious: acute and chronic pulmonary reactions, peripheral neuropathy, hepatic reactions including hepatitis, haemolytic anaemia in G6PD deficiency.
Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	● That valid informed consent was given. ● Patient name, address, date of birth, and the GP with whom they are registered. ● The symptoms present, and how long they have been present. ● **That the red flags in Appendix 1 were asked about and were absent.** ● **The answer given on known kidney disease and, where the patient is aged 60 to 64, the eGFR result relied on, its date and where it was seen (nitrofurantoin only); and the answer given on previous episodes in the last 6 and 12 months.**

	• **Which medicine was supplied and why. Where trimethoprim was supplied, the reason nitrofurantoin was unsuitable.** • Name, form, strength, dose, quantity and date of supply. • **That 48 hour safety netting was given, and what the patient was told to do.** • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Adults aged 18 and over: keep for 8 years. Under 18s: keep until the 25th birthday, or the 26th birthday where the patient was 17 when treatment finished.
Disposal	Advise the patient to return any unused medicine to a pharmacy. Do not flush or place in household waste.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	• Take one capsule twice a day for 3 days, with food or milk. Finish the course. • Your urine may go dark yellow or brown. That is expected and harmless. • Drink plenty of fluids. • Paracetamol or ibuprofen can be used for the pain if they suit you. • **Stop and seek advice if you develop new numbness, tingling or pins and needles, or if you become short of breath.**
Follow-up	As set out in the safety netting row above. 48 hours, or immediately for any red flag symptom.

Trimethoprim, second line only

Patient Group Direction for the supply of trimethoprim 200mg tablets for the treatment of uncomplicated lower urinary tract infection in non-pregnant women aged 16 to 64 years, where nitrofurantoin is unsuitable.

This arm may only be used where nitrofurantoin is unsuitable for this patient and the reason has been recorded. It is not an alternative first choice.

Healthcare professionals covered, and training

- Pharmacist registered and practising with the GPhC.
- Pharmacy technician registered and practising with the GPhC.
- All users must be familiar with the current SPC for the product supplied, the BNF, and the NICE guideline on lower urinary tract infection in women (NG109) and NICE CKS.
- All users must be able to distinguish a lower urinary tract infection from pyelonephritis, and must know the red flags in Appendix 1 without looking them up.
- All users must be competent to take a sexual history sufficient to consider a sexually transmitted infection as an alternative diagnosis, and to refer for testing without embarrassment on either side.
- All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required.
- All users should complete the Get Real Health training for this PGD before first working under it, or have completed equivalent training elsewhere, for example another provider's training for the same service or substantial recent experience of delivering it under a PGD, and must read this PGD in full before first working under it. The date the training was completed, and for equivalent training what it was, is to be recorded in the Get Real Health training record or in the employing organisation's own training record. The employing organisation remains responsible for being satisfied that each practitioner it authorises under this PGD is competent to do so.
- All users must understand capacity and consent and be aware of the Mental Capacity Act 2005.
- All users must be up to date with CPD and appraisal, and with MHRA safety alerts relating to these medicines.
- All users must hold appropriate indemnity covering this service, must work in compliance with the SOPs of their own employer, and must practise only within the bounds of their own competence.

The PGD

Indication	Treatment of uncomplicated lower urinary tract infection in a non-pregnant woman aged 16 to 64 years, where nitrofurantoin is contraindicated, not tolerated, or unavailable, and no red flag in Appendix 1 is present.
Gate on the use of this arm	**Supply trimethoprim ONLY where one of the following applies, and record which:** ● Nitrofurantoin is contraindicated for this patient, for example G6PD deficiency or a previous nitrofurantoin reaction. ● The patient has had an intolerable adverse effect on nitrofurantoin previously. ● Nitrofurantoin is not available in the pharmacy and cannot be obtained the same day. **Patient preference is NOT a reason to use this arm.** **Where trimethoprim was used in the last 3 months for any indication, do not supply it again. Resistance is likely. Refer.**
Inclusion criteria	● Female, aged 16 to 64 years inclusive. ● Two or more of: dysuria, new nocturia, urinary frequency or urgency. ● Symptoms present for 7 days or fewer. ● **No red flag in Appendix 1 present.** ● **No antibiotic already taken for this episode.** ● **Renal question answered as set out below. The eGFR requirement for patients aged 60 to 64 applies to the**

	nitrofurantoin arm only and is not a condition of this arm. ● **The gate above is satisfied and the reason recorded.** ● Able to give valid informed consent, and consent given. ● No exclusion criterion present.
Exclusion criteria	**Refer, do not supply, where any of the following applies.** ● Male. Aged under 16 or 65 and over. ● **Pregnant, possibly pregnant, or breastfeeding. Trimethoprim is a folate antagonist and is teratogenic in the first trimester. Ask; do not assume.** ● **ANY red flag in Appendix 1, INCLUDING any sign of upper urinary tract infection or pyelonephritis: fever, rigors, loin or flank pain, nausea or vomiting, visible haematuria, confusion, or appearing systemically unwell.** ● **Known kidney disease. See the renal row below. The eGFR requirement for patients aged 60 to 64 applies to the nitrofurantoin arm only.** ● **An indwelling urinary catheter, or a catheter removed within the last 7 days.** ● **Recurrent UTI, meaning 2 or more episodes in the last 6 months or 3 or more in the last 12 months.** ● **An antibiotic already taken for this same episode. One course per episode. Refer.** ● Symptoms present for more than 7 days. ● Known hypersensitivity to trimethoprim or to any excipient in the product supplied. ● Known folate deficiency, megaloblastic anaemia, or any blood dyscrasia. ● **Taking methotrexate. The combination causes profound marrow suppression and can be fatal. This is an absolute exclusion, not a caution.** ● **Taking an ACE inhibitor, an angiotensin receptor blocker, spironolactone, eplerenone, amiloride or any other potassium-sparing agent. Trimethoprim causes hyperkalaemia and the combination has been associated with sudden death in older patients.** ● Taking phenytoin, azathioprine, ciclosporin, digoxin, repaglinide or dofetilide. ● **Taking warfarin or any other coumarin anticoagulant, unless the anticoagulation service has been consulted. Trimethoprim raises the INR.** ● Known hepatic impairment. ● **Vaginal discharge, pelvic pain, intermenstrual or post-coital bleeding, or a history suggesting a sexually transmitted infection. Refer for testing.** ● Known structural or functional abnormality of the urinary tract, renal stones, or immunosuppression. ● Valid informed consent not obtained.
Renal function, how to assess it here	**Ask the patient directly: "Have you ever been told you have kidney disease, or that your kidneys do not work as well as they should?"** ● Answer YES, or the patient is under any renal follow-up: EXCLUDE. Refer. ● Answer NO and aged 16 to 59: proceed. ● **Answer NO and aged 60 to 64: proceed. No eGFR result is required for trimethoprim; the requirement for an eGFR of 45**

	mL/min or more applies to the nitrofurantoin arm only. **The patient does not know, at any age: EXCLUDE. Refer for a renal function check first.**
Cautions	Take the doses at evenly spaced intervals, roughly 12 hours apart. **Advise the patient to seek medical advice promptly for sore throat, fever, mouth ulcers, bruising or bleeding, which may indicate a blood disorder.** Advise the patient to complete the full 3 day course even if symptoms settle sooner. Encourage fluids and offer paracetamol, or ibuprofen if preferred and suitable, for pain as a separate pharmacy sale, subject to the usual checks.
Actions if the patient is excluded or declines	Explain why supply is not appropriate and what to do instead. Direct to the GP the same day where a red flag is present, or to NHS 111 or urgent care out of hours. Give the 48 hour and immediate-action advice regardless of whether anything is supplied. Document the advice given and the decision reached. Inform or refer to the GP as appropriate.
Safety netting, to be given to every patient	**48 HOURS. Tell the patient, in these terms: "If you are no better in 48 hours, or you get worse at any point, contact your GP or NHS 111 the same day. Do not wait."** **Tell the patient to seek help IMMEDIATELY, not in 48 hours, if any of the following develops:** ● A temperature, shivering or shaking uncontrollably. ● Pain in the back or side, below the ribs. ● Feeling sick or being sick. ● Blood in the urine that they can see. ● Confusion, drowsiness, or feeling very unwell in themselves. ● Any new pain or bleeding in pregnancy, if pregnancy is a possibility.

The medicine

Name, form and strength	Trimethoprim 200mg tablets.
Legal category	POM.
Dose and frequency	200mg twice daily for 3 days.
Quantity supplied	6 tablets. A 3 day course. No repeat supply under this PGD.
Route and method	Oral, with or without food.
Maximum treatment period	3 days. One course per episode. A patient still symptomatic at 48 hours is referred, not re-supplied.
Storage	Store below 25 degrees Celsius in a dry place.
Drug interactions	See the exclusion criteria. Methotrexate, potassium-sparing agents including ACE inhibitors and angiotensin receptor blockers, warfarin, phenytoin, azathioprine, ciclosporin, digoxin, repaglinide and dofetilide all interact and are handled as exclusions rather than cautions. Check the current SPC and BNF against the patient's full medicine list.
Adverse effects	Common: nausea, vomiting, diarrhoea, headache, rash, pruritus. Uncommon: hyperkalaemia, raised creatinine. Rare but serious: blood dyscrasias including megaloblastic anaemia, agranulocytosis and thrombocytopenia, severe cutaneous reactions including Stevens-Johnson syndrome, aseptic meningitis, liver enzyme abnormalities.

Yellow Card reporting	Report suspected adverse reactions via https://yellowcard.mhra.gov.uk and inform the GP.
Records to be kept	• That valid informed consent was given. • Patient name, address, date of birth, and the GP with whom they are registered. • The symptoms present, and how long they have been present. • **That the red flags in Appendix 1 were asked about and were absent.** • **The answer given on known kidney disease (no eGFR result is required for trimethoprim); and the answer given on previous episodes in the last 6 and 12 months.** • **Which medicine was supplied and why. Where trimethoprim was supplied, the reason nitrofurantoin was unsuitable.** • Name, form, strength, dose, quantity and date of supply. • **That 48 hour safety netting was given, and what the patient was told to do.** • Name and registration number of the healthcare professional supplying. • Advice given, including advice given if the patient is excluded or declines. • Details of any adverse drug reactions and the actions taken. • That the medicine was supplied under this PGD. Records signed, dated, legible and contemporaneous. Electronic records are acceptable. Adults aged 18 and over: keep for 8 years. Under 18s: keep until the 25th birthday, or the 26th birthday where the patient was 17 when treatment finished.
Disposal	Advise the patient to return any unused medicine to a pharmacy. Do not flush or place in household waste.

Information for the patient

Written information	Supply the patient information leaflet provided with the product.
Counselling	• Take one tablet twice a day for 3 days, about 12 hours apart. Finish the course. • Drink plenty of fluids. • Paracetamol or ibuprofen can be used for the pain if they suit you. • **Seek advice promptly if you get a sore throat, fever, mouth ulcers, unusual bruising or bleeding.**
Follow-up	As set out in the safety netting row above. 48 hours, or immediately for any red flag symptom.

Appendix 1: Red flags. Ask every patient, before supplying anything

If ANY of these is present, do not supply. Refer the same day.

Fever, rigors, or shivering	• Suggests the infection has reached the kidney or the bloodstream. • A lower UTI does not cause fever. Refer.
Loin or flank pain, or back pain below the ribs	• Suggests pyelonephritis. • Refer the same day. A 3 day course will not treat it.
Nausea or vomiting	• Suggests upper tract involvement, and also means an oral antibiotic may not be absorbed.
Visible blood in the urine	• Refer. Visible haematuria requires investigation in its own right, and not only for infection. • Dipstick-only (non-visible) haematuria in a symptomatic woman is not by itself a red flag.
Confusion, new drowsiness, or feeling very unwell	• Possible sepsis. Refer urgently. • Out of hours, direct to NHS 111 or urgent care rather than waiting for the surgery.
Pregnancy, or pregnancy possible	• Refer. UTI in pregnancy carries risk to the pregnancy and needs a different antibiotic choice and a test of cure.
Symptoms for more than 7 days	• Refer for assessment and culture rather than empirical treatment.
Already treated for this episode	• One course per episode. Treatment failure needs culture, not a second guess. • Refer.
Catheter in place, or removed in the last 7 days	• Not uncomplicated UTI. Refer.
Recurrent UTI	• 2 or more episodes in 6 months, or 3 or more in 12 months. • Refer for investigation, whatever today's symptoms are.

Appendix 2: Key references

- NICE NG109: Urinary tract infection (lower), antimicrobial prescribing.
- NICE Clinical Knowledge Summary: Urinary tract infection (lower), women.
- Summary of Product Characteristics for nitrofurantoin 100mg modified release capsules and trimethoprim 200mg tablets, current versions.
- British National Formulary.
- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions.

Authorisation

Version	v010
Supersedes	Version 009, 14 September 2026
Valid from date	30 September 2026
Expiry date	31 July 2027

Authorised on behalf of Get Real Health by the Medical Director and the Head Pharmacist. Their signatures for this version are on the signed authorisation page at the end of this document.	

Adoption by the employing organisation

To be completed by the adopting pharmacy. These signatories are the adopting organisation own signatories and must not be pre-filled by Get Real Health.

Superintendent / clinical lead	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....
Professional group signatory	Name: ..... Job title and organisation: ..... Signature: ..... Date: .....

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Change history to version 003

Later versions are recorded in the Version and change record at the end of this document.

Version	v003
Date	8 September 2026
Changes	● The practitioner Agreement to practise page, the premises block and the practitioner signature table are restored. Every version 001 document carried them; no document produced by the current generator did, through one omission in one function, which left 15 live documents with no page for an individual practitioner to sign. Raised as blocking by an adopting pharmacy: NICE MPG2 expects a record of the individuals authorised to work under a PGD. The standard governance requirements are restored at the same time: SPC and BNF familiarity, MHRA safety alerts, CPD and appraisal, indemnity, and capacity and consent, all of which were in version 001 and were lost in the rewrite. This version changes no clinical content. ● Full clinical review and reissue. The clinically significant changes are listed below; re-read the document before working to it. ● Safety netting is 48 hours, with a named list of symptoms requiring immediate action. ● Nitrofurantoin is first line. Trimethoprim may be supplied only where nitrofurantoin is unsuitable, and the reason must be recorded. ● Renal assessment is by direct question about known kidney disease, with exclusion where the answer is unknown in a patient aged 60 to 64. ● New exclusions in both arms: indwelling or recently removed urinary catheter; recurrent UTI, defined as 2 or more episodes in 6 months or 3 or more in 12 months; an antibiotic already taken for this episode; visible haematuria; male patients; and a history suggesting a sexually transmitted infection. ● Upper urinary tract infection is now an exclusion in the trimethoprim arm as well as the nitrofurantoin arm. ● G6PD deficiency is an exclusion in the nitrofurantoin arm. ● Trimethoprim interactions are handled as exclusions rather than cautions, and now cover potassium-sparing agents including ACE inhibitors and angiotensin receptor blockers, warfarin, phenytoin, azathioprine, ciclosporin, digoxin, repaglinide and dofetilide as well as methotrexate. Trimethoprim used within the last 3 months bars this arm. ● One course per episode. A patient still symptomatic at 48 hours is referred, not re-supplied. ● Red flags are collected in Appendix 1 and must be asked before any supply, with a record that they were asked and absent.

Previous versions

001, 25 June 2025	Development and issue of new PGD. Nitrofurantoin and trimethoprim arms, women aged 16 to 64.

Standard requirements for every practitioner working under this PGD

All users must be familiar with the current Summary of Product Characteristics for every product named in this PGD, and with current BNF and national guidance for the condition.

All users must be up to date with MHRA drug safety alerts and recalls relevant to these products.

All users must be up to date with their CPD requirements and appraisal.

All users must hold appropriate professional indemnity covering this service.

All users must understand capacity and consent, including the Mental Capacity Act 2005, and must be able to assess consent in children and young people where this PGD covers them.

Agreement to practise by the registered healthcare professional

By signing below you confirm that you agree to the contents of this PGD and that you will work within it. A PGD does not remove the inherent professional obligations or accountability of the individual practitioner. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of the pharmacy or clinic premises to which this PGD relates

Premises name	.....
Address	.... Postcode: .....
ODS code, if applicable	.....

Registered healthcare professionals authorised to work under this PGD at those premises

The employing organisation must keep this page, or an equivalent local register, and must be able to produce it on request. A practitioner who has not signed against the current version is not authorised to work under it.

Name of healthcare professional	Job title | Registration number | Signature | Date
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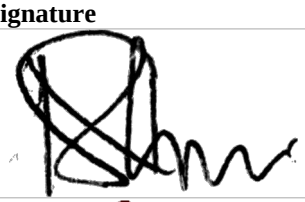
Version and change record
Version: v010
Issued: 30 September 2026
Supersedes: Version 009, 14 September 2026
Valid from: 30 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Training before first use, applied to every Get Real Health PGD on 30 September 2026: the staff section now states that each practitioner should complete the Get Real Health training for this PGD, or equivalent training elsewhere, before first working under it, must read the PGD in full before first use, and that the date of completion is recorded. Training is recorded; it is not a condition of access, and the employing organisation remains responsible for being satisfied of competence. No clinical content changes.

Previous version records
Version: v009
Issued: 14 September 2026
Supersedes: Version 008, 11 September 2026
Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.
Previous version records
Version: v008
Issued: 11 September 2026
Supersedes: Version 007, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision, joint sign-off 11 September 2026: the eGFR gate for women aged 60 to 64 (a result of 45 mL/min or more, dated within the last 12 months, seen by the pharmacist and recorded) now applies to the nitrofurantoin arm only, and the nitrofurantoin inclusion, exclusion, renal and records rows say so; in the trimethoprim arm the renal question stands as before without the eGFR requirement (YES or under renal follow-up: exclude; NO: proceed at any age from 16 to 64; does not know: exclude and refer for a renal function check first), stated in its own inclusion, exclusion, renal and records rows, and no new rule is added; the consultation tool asks for the eGFR result, and stops without one, only when nitrofurantoin is the medicine selected.
Joint sign-off by the Medical Director and the Head Pharmacist, 11 September 2026: the drafted texts of version 007 were reviewed together and confirmed, with the change above.
Previous version records
Version: v007
Issued: 11 September 2026
Supersedes: Version 006, 11 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Decision 43: a woman aged 60 to 64 who answers NO to the kidney question now proceeds where an eGFR of 45 mL/min or more, dated within the last 12 months, has been seen by the pharmacist and the result, its date and where it was seen are recorded; the figure is stated in the renal row, the inclusion and exclusion criteria and the records row of both arms, and a patient with no such result is still excluded and referred for a renal function check first.
Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.
Previous version records
Version: v006
Issued: 11 September 2026
Supersedes: Version 005, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Nitrofurantoin indication row now lists the same symptoms as the inclusion row: two or more of dysuria, new nocturia, frequency or urgency.
Trimethoprim inclusion criteria now carry the no antibiotic already taken and renal question lines that its exclusion list already relied on.
Analgesia caution carries the NICE suitability caveat for ibuprofen in both arms.
Records in both arms include the outcome of the renal question for patients aged 60 to 64.
Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.
Previous version records
Version: v005
Issued: 11 September 2026
Supersedes: Version 004, 10 September 2026
Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.
Changes in this version:
Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.
Previous version records
Version: v004
Issued: 10 September 2026
Supersedes: Version 003, 9 September 2026
Change: the summary of the governing guidance is added as part 2 of the document, between the cover and the PGD, which is the house format for every Get Real Health PGD. It is summarised from the source it names and adds nothing to it. Nothing else in this document is altered, and this is not a clinical review of the remainder.

Signed on behalf of Get Real Health

Version v010, 30 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		30 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		30 September 2026

Both authorising signatories reviewed this version together on 30 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.